
mALT
mALT
Pat & Sal's
COLE
STACY
PAT
ERN
Doppler
School Function
HELICO
Cole
Stacy
Pat
Ern

1.1 - Upper Abdominal Pain DDx

1) Clutching upper abdomen:
Upper abdominal pain can be further categorized based on pain location (RUQ, LUQ, or epigastric)

2) Green face:
Upper abdominal pain may be accompanied by **nausea and/or vomiting**

3) Liver-shaped play area on RIGHT:
Pain from the **LIVER** is 2/2 stretching of **liver capsule** → produces pain localized to **RUQ (RUQ / Liver)**

4) Alarm clock within liver area:
Acute Liver Disease (acute hepatitis, acute liver failure, etc) commonly presents with **RUQ pain (RUQ / Liver / Acute Hepatocellular)**

5) Viral capsid keychain:
Acute Liver Disease can be caused by **viral hepatitis**, excessive alcohol, acetaminophen toxicity, etc) **(RUQ / Liver / Acute Hepatocellular)**

6) Beer bottle in liver area:
Acute Liver Disease can be caused by viral hepatitis, **excessive alcohol**, acetaminophen toxicity, etc) **(RUQ / Liver / Acute Hepatocellular)**

7) “ICY” spray:
Acute Liver Disease can be caused by viral hepatitis, excessive alcohol, **acetaminophen toxicity**, etc) **(RUQ / Liver / Acute Hepatocellular)**

8) Yellow billy goat in liver area:
Acute Liver Disease often presents with **jaundice** 2/2 conjugated hyperbilirubinemia **(RUQ / Liver / Acute Hepatocellular)**

9) “Pat & Sal’s”:
Acute Liver Disease presents with a **HEPATOCELLULAR lab pattern** = predominantly elevated AST/ALT +/- mildly elevated alkaline phosphatase **(RUQ / Liver / Acute Hepatocellular)**

10) Elevated toASTing “mALT” glasses:
Acute Liver Disease presents with a HEPATOCELLULAR lab pattern = **predominantly elevated AST/ALT +/-** mildly elevated alkaline phosphatase **(RUQ / Liver / Acute Hepatocellular)**

11) “ChALK” bucket on the ground:
Acute Liver Disease presents with a HEPATOCELLULAR lab pattern = predominantly elevated AST/ALT +/- **mildly elevated alkaline phosphatase (RUQ / Liver / Acute Hepatocellular)**

12) Collared billy goat near “Pat & Sal’s”:
As part of HEPATOCELLULAR lab pattern, Acute Liver Disease often presents with **conjugated hyperbilirubinemia (RUQ / Liver / Acute Hepatocellular)**

13) Red ball in liver area:
Liver Abscess (pyogenic or amebic) can present with RUQ pain **(RUQ / Liver / Abscess)**

14) Bacterial logo over abdomen:
Pyogenic Liver Abscess is caused by **bacteria**, usually 2/2 spread from abdominal source (peritonitis or biliary infection) **(RUQ / Liver / Abscess / Pyogenic)**

15) Amoeba backpack:
Amebic Liver Abscess is caused by the protozoan *Entamoeba histolytica* **(RUQ / Liver / Abscess / Amebic)**

16) Textbook with map:
In the US, Amebic Liver Abscess occurs primarily in **travelers or migrants** from endemic areas (Mexico, Central & South America, Africa, India) **(RUQ / Liver / Abscess / Amebic)**

17) Flame bandana near red ball in liver area:
Liver Abscess (pyogenic or amebic) presents with **fever (RUQ / Liver / Abscess)**

18) White birds near red ball:
Liver Abscess (pyogenic or amebic) presents with **leukocytosis (RUQ / Liver / Abscess)**

19) Blue spring & porthole near alarm clock:
Acute Portal Vein Thrombosis can present with RUQ pain **(RUQ / Liver / Acute PVT)**

20) Flame bandana on porthole kid:
Acute Portal Vein Thrombosis often presents with **fever (RUQ / Liver / Acute PVT)**

21) Liver rock:
Cirrhosis is a risk factor for Acute Portal Vein Thrombosis **(RUQ / Liver / Acute PVT)**

22) Pile of sticks:
A **hypercoagulable state** is a risk factor for Acute Portal Vein Thrombosis **(RUQ / Liver /Acute PVT)**

23) Throwing a FIT:
Fitz-Hugh-Curtis Syndrome (aka perihepatitis; inflammation of liver capsule and adjacent peritoneum) presents with RUQ pain **(RUQ / Liver / Fitz-Hugh-Curtis)**

24) Red lining of dress:
Fitz-Hugh-Curtis Syndrome (**perihepatitis**; inflammation of liver capsule and adjacent peritoneum) presents with RUQ pain **(RUQ / Liver / Fitz-Hugh-Curtis)**

25) Flame notebook in uterus backpack:
Fitz-Hugh-Curtis Syndrome is a clinical manifestation of **Pelvic Inflammatory Disease (PID)** → affects sexually active young woman **(RUQ / Liver / Fitz-Hugh-Curtis)**

25) Flame notebook in uterus backpack:
Fitz-Hugh-Curtis Syndrome is a clinical manifestation of **Pelvic Inflammatory Disease (PID)** → affects sexually active young woman **(RUQ / Liver / Fitz-Hugh-Curtis)**

26) Worse with deep breaths:
Fitz-Hugh-Curtis Syndrome causes severe RUQ pain with a **pleuritic quality** (i.e. aggravated by deep inspiration) +/- referred pain to R shoulder **(RUQ / Liver / Fitz-Hugh-Curtis)**

27) Getting pecked on R shoulder:
Fitz-Hugh-Curtis Syndrome causes severe RUQ pain with a pleuritic quality (i.e. aggravated by deep inspiration) +/- **referred pain to R shoulder (RUQ / Liver / Fitz-Hugh-Curtis)**

28) Flame bandana on tantrum girl:
Fitz-Hugh-Curtis Syndrome may present with **fever (RUQ / Liver / Fitz-Hugh-Curtis)**

29) White bird pestering tantrum girl:
Fitz-Hugh-Curtis Syndrome may present with**leukocytosis (RUQ / Liver / Fitz-Hugh-Curtis)**

30) Round green play area on RIGHT:
Pain from **GALLBLADDER** (Biliary Colic, Acute Cholecystitis, etc) is typically located in **RUQ/epigastric area** +/- referred pain to R shoulder or R subscapular region of back **(RUQ / Gallbladder)**

31) Painfully tight leash around shoulder:
Pain from GALLBLADDER (Biliary Colic, Acute Cholecystitis, etc) is typically located in RUQ/epigastric area +/- **referred pain to R shoulder or R subscapular region of back (RUQ / Gallbladder)**

32) Green marble pouch:
Biliary Colic is the most common symptom of **gallstones (RUQ / Gallbladder / Biliary Colic)**

33) Collie:
Biliary Colic is the most common symptom of gallstones **(RUQ / Gallbladder / Biliary Colic)**

34) Collie's leash inducing temporary discomfort:
Biliary Colic presents with dull discomfort lasting 30 mins to a few hours (6 hours max) (RUQ / Gallbladder / Biliary Colic)

35) Fatty pizza:
Biliary Colic is commonly triggered by a **fatty meal (RUQ / Gallbladder / Biliary Colic)**

36) Sleepy cap:
Biliary Colic commonly occurs **at night (RUQ / Gallbladder / Biliary Colic)**

37) Flame-patterned marble bag:
Acute cholecystitis (inflammation of the gallbladder) commonly presents with RUQ pain **(RUQ / Gallbladder / Acute Cholecystitis)**

38) Collie statue:
Acute Cholecystitis, Choledocholithiasis & Acute Cholangitis present with pain **more prolonged than Biliary Colic** (> 6 hours) **(RUQ / Gallbladder & Bile Ducts)**

39) Cheese pizza:
Acute Cholecystitis may be triggered by a fatty meal (often > 1 hour prior to onset of symptoms) **(RUQ / Gallbladder / Acute Cholecystitis)**

40) Gasping from hit to RUQ:
Murphy's sign (abrupt cessation of inspiration on deep palpation of gallbladder fossa) suggests Acute Cholecystitis **(RUQ / Gallbladder / Acute Cholecystitis)**

41) Flame bandana near flame marble bag:
Acute cholecystitis commonly presents with **fever (RUQ / Gallbladder / Acute Cholecystitis)**

42) White bird near flame marble bag:
Acute cholecystitis commonly presents with**leukocytosis (RUQ / Gallbladder / Acute Cholecystitis)**

43) Rectangular green play area on RIGHT:
Pain from EXTRAhepatic BILE DUCTS (Choledocholithiasis, Acute Cholangitis, Sphincter of Oddi Dysfunction, etc) is typically localized to RUQ/epigastric area **(RUQ / Bile Ducts)**

44) Marble at base of biliary tree:
Choledocholithiasis (gallstones in the common bile duct [CBD]) presents with RUQ pain **(RUQ / Bile Ducts / Choledocholithiasis)**

45) Yellow billy goat in green play area:
Choledocholithiasis & Acute Cholangitis present with jaundice 2/2 cholestasis **(RUQ / Bile Ducts)**

46) "Cole-Stacy Pat-Ern" near green play area:
Choledocholithiasis & Acute Cholangitis present with a **CHOLESTATIC pattern** of labs**(RUQ / Bile Ducts)**

7) Elevated "chALK" bucket:
CHOLESTATIC pattern of labs = **markedly elevated alkaline phosphatase** +/- mildly elevated AST/ALT; elevated GGT, conjugated hyperbilirubinemia

48) ToASTing small "mALT" shakes:
CHOLESTATIC pattern of labs = markedly elevated alkaline phosphatase +/- **mildly elevated AST/ALT** ; elevated GGT, conjugated hyperbilirubinemia

49) Raised GGT gadget knife:
CHOLESTATIC pattern of labs = markedly elevated alkaline phosphatase +/- mildly elevated AST/ALT; **elevated GGT**, conjugated hyperbilirubinemia

50) Collared billy goat near “Cole-Stacy Pat-Ern”:
CHOLESTATIC pattern of labs = markedly elevated alkaline phosphatase +/- mildly elevated AST/ALT; elevated GGT, **conjugated hyperbilirubinemia**

51) Flame in green play area:
Acute Cholangitis (inflammation and bacterial infection of bile ducts) presents with RUQ pain **(RUQ / Bile Ducts / Acute Cholangitis)**

52) Triangle bell with short coat:
Acute cholangitis classically presents with **Charcot’s Triad** (RUQ pain, jaundice, fever) **(RUQ / Bile Ducts / Acute Cholangitis)**

53) Yellow coat:
Acute cholangitis classically presents with Charcot’s Triad (RUQ pain, **jaundice**, fever)**(RUQ / Bile Ducts / Acute Cholangitis)**

54) Flame bandana in green play area:
Acute cholangitis presents with Charcot’s Triad (RUQ pain, jaundice, **fever**) **(RUQ / Bile Ducts / Acute Cholangitis)**

55) White bird on triangle:
Acute cholangitis presents with leukocytosis **(RUQ / Bile Ducts / Acute Cholangitis)**

56) Malfunctioning green squirt gun near collie:
Sphincter of Oddi dysfunction (sphincter stenosis or dyskinesia) presents with RUQ pain similar to biliary colic (lasts 30 mins to a few hours) **(RUQ / Bile Ducts / SOD)**

57) “More Fun”:
In Sphincter of Oddi dysfunction, pain may be aggravated by **morphine (RUQ / Bile Ducts / SOD)**

58) Faint transient “Cole-Stacy Pat-Ern”:
In Sphincter of Oddi dysfunction, **alkaline phosphatase** and/or **AST/ALT**may be elevated during pain episodes → labs normalize between attacks **(RUQ / Bile Ducts / SOD)**

59) Bullhorn towering over RIGHT scene:
Work-up for RUQ pain typically includes imaging with **ultrasound (RUQ)**

60) Bullhorn casting a shadow:
In patients with Biliary Colic, ultrasound typically demonstrates gallstones in the gallbladder → look for **hyperechoic foci that cast a shadow (RUQ / Gallbladder / Biliary Colic)**

61) Bullhorn with thick rim:
In Acute Cholecystitis, ultrasound typically demonstrates **gallbladder wall thickening (RUQ / Gallbladder / Acute Cholecystitis)**

62) Marble within bullhorn range:
In Choledocholithasis, ultrasound may demonstrate a **stone in the common bile duct (RUQ / Choledocholithiasis)**

63) Dilated green windsock:
On ultrasound, **dilation of the common bile duct** suggests biliary obstruction (may be 2/2 choledocholithiasis, acute cholangitis, sphincter of oddi dysfunction, malignant obstruction, etc)

64) Bullhorn with round ball:
In Liver Abscess (pyogenic or amebic), ultrasound typically demonstrates a **round mass (RUQ / Liver / Abscess)**

65) “Doppler” with colored sound lines:
In Acute Portal Vein Thrombosis, **doppler ultrasound** demonstrates absence of flow in portal vein **(RUQ / Liver / Acute PVT)**

66) Oversized liver play area within bullhorn range:
In Acute Liver Disease, ultrasound may demonstrate **hepatomegaly (RUQ / Liver / Acute Hepatocellular)**

67) Splenic sandbox on LEFT:
Pain from the **SPLEEN** is typically localized to **LUQ (LUQ / Spleen)**

68) LARGE spleen-shaped sandbox:
Splenomegaly can present with LUQ pain/discomfort **(LUQ / Spleen / Splenomegaly)**

69) Pushing away lunchbox:
Splenomegaly can present with early satiety (LUQ / Spleen / Splenomegaly)

70) Liver rock in sandbox:
Splenomegaly can be caused by **cirrhosis**, lymphoma/leukemia, myeloproliferative disorders, infection, hemolytic anemia, etc. **(LUQ / Spleen / Splenomegaly)**

71) Chessboard graffiti in sandbox:
Splenomegaly can be caused by cirrhosis, **lymphoma**/leukemia, myeloproliferative disorders, infection, hemolytic anemia, etc. **(LUQ / Spleen / Splenomegaly)**

72) Palpating sandbox:
An enlarged spleen is usually **palpable** on physical exam **(LUQ / Spleen / Splenomegaly)**

73) Skull mask:
Splenic Infarct (occlusion of splenic artery or one of its branches) typically presents with severe LUQ pain **(LUQ / Spleen / Infarct)**

74) Playing with sticks:
A hypercoagulable state is a risk factor for Splenic Infarction **(LUQ / Spleen / Infarct)**

75) Afib ECG shirt:
Atrial fibrillation is a risk factor for Splenic Infarction **(LUQ / Spleen / Infarct)**

76) Red ball in splenic sandbox:
Splenic Abscess presents with LUQ pain **(LUQ / Spleen / Abscess)**

77) Heart-shaped flame lantern:
Splenic Abscess can be caused by hematogenous spread of bacteria from**infective endocarditis (LUQ / Spleen / Abscess)**

78) Flame bandana near red ball in sandbox:
Splenic Abscess presents with **fever** (may persist despite antibiotic therapy) **(LUQ / Spleen / Abscess)**

79) White bird targeted by red ball:
Splenic Abscess presents with **leukocytosis (LUQ / Spleen / Abscess)**

80) Bullhorn near splenic sandbox:
Work-up of LUQ pain typically includes imaging (**ultrasound** or CT scan) **(LUQ)**

81) CT cat near splenic sandbox:
Work-up of LUQ pain typically includes imaging (ultrasound or **CT scan**) **(LUQ)**

82) Stomach-shaped area in MIDLINE:
Pain from the STOMACH typically presents as DYSPEPSIA (EPIGASTRIC pain +/- other upper GI symptoms); cardiac or esophageal pain can mimic this presentation**(Epigastric / Dyspepsia)**

83) Full stomach-shaped trash bin:
DYSPEPSIA = EPIGASTRIC pain +/- other “upper GI symptoms” (postprandial **epigastric fullness**, heartburn, nausea)**(Epigastric / Dyspepsia)**

84) Lunch remnants near full stomach bin:
DYSPEPSIA = EPIGASTRIC pain +/- other “upper GI symptoms” (**postprandial** epigastric fullness, heartburn, nausea) **(Epigastric / Dyspepsia)**

85) Flame logo over chest:
DYSPEPSIA = EPIGASTRIC pain +/- other “upper GI symptoms” (postprandial epigastric fullness, **heartburn**, nausea) **(Epigastric / Dyspepsia)**

86) Angina anvil:
Cardiac ischemia [stable angina or ACS (unstable angina, NSTEMI, STEMI)] can present with dyspepsia (considered an “atypical” presentation of ischemia) **(Epigastric / Dyspepsia / Cardiac Ischemia)**

87) Old lady:
Atypical presentations of cardiac ischemia are most common in **women, the elderly**, and diabetics **(Epigastric / Dyspepsia / Cardiac Ischemia)**

88) Candy jar:
Atypical presentations of cardiac ischemia are most common in women, the elderly, and **diabetics (Epigastric / Dyspepsia / Cardiac Ischemia)**

89) Coronary crown:
Risk factors for cardiac ischemia = **known coronary artery disease (CAD)** OR **multiple risk factors for CAD** (diabetes, hypertension, hyperlipidemia, smoking, etc) **(Epigastric / Dyspepsia / Cardiac Ischemia)**

90) Blowing puff of air:
Dyspnea or exertional symptoms increase suspicion for cardiac disease **(Epigastric / Dyspepsia / Cardiac Ischemia)**

91) Difficulty exercising:
Dyspnea or **exertional symptoms** increase suspicion for cardiac disease **(Epigastric / Dyspepsia / Cardiac Ischemia)**

92) ECG apron:
Work-up for cardiac ischemia includes **ECG**; add serum troponins if ACS suspected **(Epigastric / Dyspepsia / Cardiac Ischemia)**

93) T-bone steak:
Work-up for cardiac ischemia includes ECG; add **serum troponins** if ACS suspected **(Epigastric / Dyspepsia / Cardiac Ischemia)**

94) Overflowing acidic soda:
Gastroesophageal reflux disease (GERD) can present with dyspepsia **(Epigastric / Dyspepsia / GERD)**

95) Classic flame logo over chest:
Classic GERD symptoms = **heartburn (retrosternal/epigastric burning pain)** & regurgitation **(Epigastric / Dyspepsia / GERD)**

96) Acidic soda dripping into mouth:
Classic GERD symptoms = heartburn (retrosternal/epigastric burning pain) & **regurgitation (Epigastric / Dyspepsia / GERD)**

97) Lunch remnants near overflowing soda:
GERD symptoms tend to occur **after meals (i.e postprandial) (Epigastric / Dyspepsia / GERD)**

98) Lying down wearing sleepy cap:
GERD symptoms are common **at night when lying supine (Epigastric / Dyspepsia / GERD)**

99) Chugging tablets:
GERD symptoms are typically **relieved with antacids (Epigastric / Dyspepsia / GERD)**

100) Coughing kid:
GERD can cause **chronic cough** 2/2 laryngeal irritation **(Epigastric / Dyspepsia / GERD)**

101) Black tarry fluid leaking down STOMACH area:
Structural disorders of the STOMACH/upper duodenum (gastropathy, gastritis, peptic ulcer disease, gastric cancer, etc) can present with **Upper GI Bleeding (Epigastric / Dyspepsia)**

102) Black tarry fluid:
Symptoms of upper GI bleed = **melena (black tarry stool)** and/or hematemesis (coffee-ground emesis or red blood) **(Epigastric / Dyspepsia)**

103) Coffee ground-like dirt:
Symptoms of upper GI bleed = melena (black tarry stool) and/or **hematemesis** (**coffee-ground emesis** or red blood) **(Epigastric / Dyspepsia)**

104) Bringing-up red fluid:
Symptoms of upper GI bleed = melena (black tarry stool) and/or **hematemesis** (coffee-ground emesis or **red blood**) **(Epigastric / Dyspepsia)**

105) Cracked stomach beaker:
Gastropathy (gastric mucosal damage WITHOUT inflammation) can present with dyspepsia **(Epigastric / Dyspepsia / Gastropathy)**

106) Anti-inflammatory fire extinguisher:
Gastropathy can be caused by ingestion of **NSAIDs**, alcohol, or chemotherapy (2/2 direct mucosal injury) **(Epigastric / Dyspepsia / Gastropathy)**

107) ETOH beaker:
Gastropathy can be caused by ingestion of NSAIDs, **alcohol**, or chemotherapy (2/2 direct mucosal injury) **(Epigastric / Dyspepsia / Gastropathy)**

108) Chemistry set:
Gastropathy can be caused by ingestion of NSAIDs, alcohol, or **chemotherapy** (2/2 direct mucosal injury) **(Epigastric / Dyspepsia / Gastropathy)**

109) Cocoa mug:
Gastropathy can be caused by **cocaine** use (vasoconstriction → mucosal ischemia) **(Epigastric / Dyspepsia / Gastropathy)**

110) Stomach beaker over flame:
Gastritis (inflammation of gastric mucosa) can present with dyspepsia **(Epigastric / Dyspepsia / Gastritis)**

11) Toy “HELICO” helicopter:
***Helicobacter pylori* infection** = risk factor for gastritis, peptic ulcer disease (both gastric and duodenal), and gastric cancer **(Epigastric / Dyspepsia)**

112) Antibody-shaped tongs:
Gastritis can be caused by **autoimmune disease** (i.e. autoimmune metaplastic atrophic gastritis) **(Epigastric / Dyspepsia / Gastritis)**

113) Deep erosion:
Peptic ulcer disease (PUD) includes gastric and duodenal ulcers (deep defects extending through muscularis mucosa); both types commonly present with dyspepsia **(Epigastric / Dyspepsia / PUD)**

114) Lunch remnants near deep erosion:
GASTRIC ulcer pain is typically **postprandial (Epigastric / Dyspepsia / PUD)**

115) Empty lunchbox near deep erosion:
DUODENAL ulcer pain typically occurs on an **empty stomach** → improves with meals **(Epigastric / Dyspepsia / PUD)**

116) Anti-inflammatory fire extinguisher next to deep erosion:
NSAID use is a risk factor for PUD (both gastric and duodenal ulcers) **(Epigastric / Dyspepsia / PUD)**

117) Crab toy in stomach area:
Gastric Cancer can present with dyspepsia **(Epigastric / Dyspepsia / Gastric Cancer)**

118) Old maintenance man:
In the US, Gastric Cancer occurs primarily in older patients (age ≥ 60) **(Epigastric / Dyspepsia / Gastric Cancer)**

119) Thin arms near crab toy:
Gastric Cancer commonly presents with weight loss **(Epigastric / Dyspepsia / Gastric Cancer)**

120) Falling iron tools:
Gastric Cancer can present with **Fe deficiency anemia** 2/2 chronic occult blood loss **(Epigastric / Dyspepsia / Gastric Cancer)**

121) “School Function”:
Functional Dyspepsia (dyspepsia without a structural cause) is the most common cause of dyspepsia **(Epigastric / Dyspepsia / Functional)**

122) Flashlight in mouth-shaped slide:
Work-up for dyspepsia includes **upper endoscopy (EGD)** in select patients [i.e. increased risk of gastric cancer (age > 60, significant weight loss, etc) or overt upper GI bleeding] **(Epigastric / Dyspepsia)**

123) UREASE spray bottle:
In dyspepsia patients who do not require EGD, perform non-invasive H. pylori testing (**urease breath test** or stool antigen test) **(Epigastric / Dyspepsia)**

124) Dirty brown rag:
In dyspepsia patients who do not require EGD, perform non-invasive H. pylori testing (urease breath test or **stool antigen test**) **(Epigastric / Dyspepsia)**

125) Pancreas sponge in MIDLINE:
Pain from the **PANCREAS** is typically located in **EPIGASTRIC** area +/- referred pain straight to the BACK **(Epigastric / Pancreas)**

126) Causing back pain:
Pain from the PANCREAS is typically located in EPIGASTRIC area +/- **referred pain straight to the BACK (Epigastric / Pancreas)**

127) Alarm clock near pancreas sponge:
Acute Pancreatitis presents with **ACUTE** onset of SEVERE epigastric pain radiating to the back **(Epigastric / Pancreas / Acute Pancreatitis)**

128) “I GET SMASHED”:
Causes of Acute Pancreatitis = **I**diopathic, **G**ALLSTONES, **E**THANOL, **T**rauma, **S**teroids & Shock, **M**umps & other infections, **A**utoimmune, **S**corpion stings, **H**ypertriglyceridemia & Hypercalcemia, **E**RCP, and**D**rugs **(Epigastric / Pancreas / Acute Pancreatitis)**

129) Dripping fat:
Acute Pancreatitis typically presents with **elevation of serum lipase** and/or serum amylase (> 3x upper limit of normal) **(Epigastric / Pancreas / Acute Pancreatitis)**

130) Dripping sugary juice:
Acute Pancreatitis typically presents with elevation of serum lipase and/or **serum amylase** (> 3x upper limit of normal) **(Epigastric / Pancreas / Acute Pancreatitis)**

131) “3X” face shape:
Acute Pancreatitis typically presents with elevation of serum lipase and/or serum amylase **(> 3x upper limit of normal) (Epigastric / Pancreas / Acute Pancreatitis)**

132) Grandfather clock near pancreas sponge:
Chronic Pancreatitis may present with **CHRONIC** epigastric pain radiating to the back **(Epigastric / Pancreas / Chronic Pancreatitis)**

133) Lunch remnants near grandfather clock:
Chronic Pancreatitis may initially present with **postprandial** pain attacks → pain may become constant as disease progresses **(Epigastric / Pancreas / Chronic Pancreatitis)**

134) Mud with fatty chunks:
Chronic pancreatitis often presents with **steatorrhea** (loose, fatty, foul smelling stools) 2/2 pancreatic insufficiency **(Epigastric / Pancreas / Chronic Pancreatitis)**

135) Beer bottle:
Chronic alcohol use can cause Chronic Pancreatitis **(Epigastric / Pancreas / Chronic Pancreatitis)**

136) Crab graffiti near pancreas sponge:
Exocrine **Pancreatic Cancer** may present with insidious onset of epigastric pain radiating to the back **(Epigastric / Pancreas / Cancer)**

137) Sleepy cap near cancer crab:
Pancreatic cancer pain is classically **worse at night (Epigastric / Pancreas / Cancer)**

138) Lunch remnants near cancer crab:
Pancreatic Cancer pain can be **postprandial (Epigastric / Pancreas / Cancer**

139) Thin arms near crab graffiti:
Pancreatic cancer commonly presents with **weight loss (Epigastric / Pancreas / Cancer)**

140) Yellow jumpsuit:
Pancreatic Cancer can present with **jaundice** (mass in pancreatic head → common bile duct [CBD] obstruction → cholestasis) **(Epigastric / Pancreas / Cancer)**

141) Clay-colored bird poop:
Pancreatic Cancer can present with **acholic (clay-colored) stools** and dark urine (2/2 CBD obstruction from mass in pancreatic head) **(Epigastric / Pancreas / Cancer)**

142) Dark urine bucket:
Pancreatic Cancer can present with acholic (clay-colored) stools and **dark urine** (2/2 CBD obstruction from mass in pancreatic head) **(Epigastric / Pancreas / Cancer)**

143) Green bag protruding from right side:
Pancreatic Cancer can present with **Courvoisier's sign** (non-tender **palpable gallbladder**in a jaundiced patient) (2/2 CBD obstruction from mass in pancreatic head) **(Epigastric / Pancreas / Cancer)**

144) "Cole-Stacy Pat-Ern" near pancreas sponge:
Pancreatic Cancer can present with a **CHOLESTATIC pattern** of labs (2/2 CBD obstruction from mass in pancreatic head) **(Epigastric / Pancreas / Cancer)**

145) Bullhorn near pancreas sponge:
Initial work-up of pancreatic disorders often involves **ultrasound** or CT scan **(Epigastric / Pancreas)**

146) Black cat near pancreas sponge:
Initial work-up of pancreatic disorders often involves ultrasound or **CT scan (Epigastric / Pancreas)**